Lesson 09: Putting Ethics into Practice

English Stage 4 (Psychology) · Unit 2: Research Ethics and Measurement

Adapted from Research Methods in Psychology (4th American edition) by Rajiv S. Jhangiani, I-Chant A. Chiang, Carrie Cuttler and Dana C. Leighton (CC BY-NC-SA 4.0): 3.4 Putting Ethics Into Practice; 3.5 Research Ethics (Summary).

In this lesson

You learn the practical steps of ethical research, from knowing your responsibilities to reporting results honestly, and how researchers reduce risk and deception (for example, Burger's 2009 replication of Milgram). You practise the language of procedures and advice, and write a consent form and debriefing script in plain English.

Learning goals

After this lesson you should be able to:

- Describe the steps of ethical research in order.
- Explain three ways to minimize risks, and how to minimize deception.
- Write consent and debriefing procedures that cover APA Standard 8.02a.
- Use imperatives, modal advice and sequencing, and rewrite formal English in plain English.

Key language

Term or phrase	Meaning	Example
prescreening	Any procedure used to select participants based on their characteristics, often to identify and remove those at high risk of harm.	Burger used an extensive prescreening procedure.
replication	Conducting a study again, exactly or with modifications, to see if the same results are observed.	a 2009 replication of Milgram's study
talking points	a short list of points to say, so that you can explain the study in simple everyday language	Prepare a script or set of "talking points".
debriefing	Informing participants after a study of its purpose, revealing any deception, and minimizing any harm.	Debriefing is a good time to provide additional benefits.
referral	information that directs a person to another source of help	referral information to the university counseling center
publication credit	who will be authors, and in what order	Address publication credit with your collaborators early.
as long as	on condition that	… as long as you describe the procedure, risks, and benefits

Rules and explanations

The ideas

Ethical issues arise before data collection and continue through publication. Section 3.4 gives these steps:

1. **Know and accept your responsibilities.** "Lack of awareness or misunderstanding of an ethical standard is not itself a defense" (APA Ethics Code). Know the Code and your institution's IRB procedures; ask when unsure.
2. **Identify and minimize risks.** List all risks; researchers easily underestimate them (a student who was an emergency medical technician did not realize how disturbing her crime and accident photos were). Some risks affect only some participants (crime victims answering a survey on fear of crime), so seek input from others. Then:
    - **Modify the design. Burger (2009)** replicated Milgram's study but always stopped the procedure when participants were about to give the 150-V shock: in Milgram's study the severe reactions came after this point, and most participants who gave the 150-V shock went on to the 450-V maximum. So he could compare results without causing severe stress. (His participants were just as obedient.)
    - **Prescreen.** Warn participants about sensitive content and remind them that they may withdraw; Burger also used questionnaires and an interview with a clinical psychologist to exclude high-risk participants.
    - **Protect confidentiality.** Keep consent forms separate from data, collect only the personal information you need, and let people answer personal questions in private.
3. **Identify and minimize deception**, including withholding information. If you show photos of friends as "college professors", that is deception; asking participants to *imagine* they are professors removes it. Waiting until debriefing to reveal the research question is acceptable as long as you describe the procedure, risks and benefits; you can even tell participants that you will reveal it afterwards.
4. **Weigh risks against benefits** (including what you learn as a student researcher). Minimal risk needs only a small benefit; more risk needs more benefit. Harm that is more than minor or lasts more than a short time is rarely justified.
5. **Create consent and debriefing procedures.** Check Standard 8.05; inform people when recruiting; prepare talking points; write a consent form covering Standard 8.02a. In debriefing, reveal the research question and full design; reveal any deception, apologize, explain why it was necessary and correct misconceptions; offer information or referrals (for example, counseling information after a study on domestic abuse).
6. **Get approval** with a protocol, and see IRB review as an opportunity, not an obstacle.
7. **Follow through.** Stick to the protocol and monitor reactions; Baumrind (1985) criticized Milgram for not changing his procedure after the first several participants. Agree on publication credit early, and never fabricate or alter results.

Language focus

1. Procedural advice. Section 3.4 speaks directly to the reader:

- Imperatives: "**Start by listing** all the risks, including risks of physical and psychological harm ..." "**Remember** to schedule plenty of time for the informed consent and debriefing processes."
- Advice: "you **should** seek input from a variety of people"; "**It is best to** identify and minimize *all* forms of deception"; "it **is considered acceptable to** wait until debriefing".
- Sequencing: "**Once you have identified** the risks, ...", "**First**, ... **Second**, ... **Third**, ...", "**The next step is** to get institutional approval", "**Finally**, ...".

2. Plain English. The book says to explain the study "in simple everyday language". A consent form uses *you* and *we*, short sentences and everyday words: compare the Code's "their right to decline to participate and to withdraw from the research once participation has begun" with "You do not have to take part, and you can stop at any time."

> **Note for Arabic speakers** - After *once*, *when*, *after* and *as soon as*, use the present (perfect) for the future: "Once you have identified the risks ...", not "Once you will identify ...". - Avoid long chains of clauses joined by *and*; a consent form is clearer with one idea per sentence.

Practice

1. Reading questions (section 3.4)

1. Why did the student researcher underestimate the risk of her photographs?
2. How did Burger (2009) modify Milgram's procedure, why did this make sense, and what did he find?
3. What are the book's three ways to minimize risks?
4. How could the deception in the "college professors" photo study be eliminated?
5. Why is it acceptable to wait until debriefing to reveal the research question?
6. What criticism of Milgram does the book give under "Follow Through"?

2. Book exercises (3.5; they cover the whole chapter, Lessons 07-09)

1. Practice: Imagine a study testing the effectiveness of a new drug for treating obsessive-compulsive disorder. Give a hypothetical example of an ethical issue from each cell of Table 3.1 "A Framework for Thinking About Ethical Issues in Scientific Research" that could arise in this research. (The table is the framework in Lesson 07.)
2. Discussion: It has been argued that researchers are not ethically responsible for the misinterpretation or misuse of their research by others. Do you agree? Why or why not?
3. Practice: Read the Nuremberg Code, the Belmont Report, and *Standard 8* of the APA Ethics Code. List five specific similarities and five specific differences among them.
4. Discussion: In a study on the effects of disgust on moral judgment, participants were asked to judge the morality of disgusting acts, including people eating a dead pet and passionate kissing between a brother and sister (Haidt, Koller, & Dias, 1993). If you were on the IRB that reviewed

this protocol, what concerns would you have with it? Refer to the appropriate sections of the APA Ethics Code.

5. Discussion: How could you conduct a study on the extent to which people obey authority in a way that minimizes risks and deception as much as possible? (Note: Such a study would not have to look at all like Milgram's.)
6. Practice: Find a study in a professional journal and create a consent form for that study. Be sure to include all the information in Standard 8.02. (Do this in the Writing task, with your own planned study.)

3. Language A. Rewrite in plain English for a consent form.

1. Participants have the right to decline to participate and to withdraw from the research once participation has begun.
2. Signed consent forms will be stored separately from the data so that no individual's name can be linked to their responses.

B. Put these steps in the book's order and write them as one paragraph with *First*, *Once ...*, *The next step is* and *Finally*: get approval; follow through; weigh the risks against the benefits; create consent and debriefing procedures; identify and minimize risks.

Speak with your tutor

- "Let me explain why Burger's 2009 replication reduced risk. Correct my explanation and my sequencing words."
- "In PSYC 110 Lecture 15 (Morality), Professor Bloom presents Haidt's stories, such as a family that eats its dead dog. Play the IRB chair and question me about the risks of using such stories."
- "Role-play a participant in my sleep and memory study. I will explain it with my talking points; ask what a real participant would ask. Then let me debrief you."

Writing task

Write the **consent form and debriefing talking points** for your study (sleep and memory in university students, or your own topic): 250-400 words in total.

Part A, consent form: a title; the eight points of Standard 8.02a (for the default study: the procedure, that is, learning 40 word pairs, then either a nap opportunity of up to 90 minutes or quiet rest for the same time, decided at random, then a recall test, and how long it all takes; risks such as tiredness, boredom or the stress of a memory test; course credit with an alternative activity); if relevant, that some information will be explained at the end; a signature and date line.

Part B, debriefing talking points (5-6 bullets): the research question and full design; any deception, with an apology and the reason; correcting misconceptions; useful information or referrals; thanks.

If you work on your own topic, write the same documents for your study.

Self-editing checklist:

- Does Part A cover all eight points of Standard 8.02a?

- Is it plain English, with *you*, *we* and short sentences?
- Are the risks specific and honest?
- If information is withheld, does the form say so?
- Does Part B reveal the full design and any deception?
- Did I replace every formal phrase a participant might not understand?

Paste your draft into the chat and ask your tutor for feedback. Then rewrite it.

Watch online

- The Nuremberg Code, the Belmont Report and other codes (Exercise 3): https://www.hhs.gov/ohrp/international/ethical-codes-and-research-standards/index.html
- The full APA Ethics Code (Exercises 3, 4 and 6): https://www.apa.org/ethics/code
- Haidt, Koller and Dias (1993), the study in Exercise 4 (may need library access): https://doi.org/10.1037/0022-3514.65.4.613

Answer key

1. Reading questions

1. She was an emergency medical technician, so she greatly underestimated how disturbing the images were to most people.
2. He always stopped when participants were about to give the 150-V shock. In Milgram's study the severe reactions came after this point, and most who gave the 150-V shock continued to the maximum, so Burger could compare results and estimate how many would have continued, without severe stress. His participants were just as obedient.
3. Modify the research design; prescreen to identify and eliminate high-risk participants; take active steps to maintain confidentiality.
4. Tell participants to *imagine* that the photos are of college professors and to rate them *as if* they were.
5. It is unlikely to affect people's decision to take part, and knowing it could invalidate the results (participants might rate older and younger "professors" differently, or carefully the same).
6. He did not know in advance about the severe reactions, but he knew after the first several participants and should have changed the procedure then (Baumrind, 1985).

2. Book exercises

1. (example answer; many answers are possible)

	Participants	**Scientific community**	**Society**
Risks and benefits	side effects vs. relief of symptoms	a poor design wastes resources vs. new knowledge	the drug is overprescribed vs. a new treatment
Integrity	honesty that the drug is experimental and how groups are assigned (8.02b)	reporting results honestly even if the drug fails	a researcher who gains financially from the drug
Justice	offering the drug to the control group if it works	fair authorship credit	not recruiting only poor or institutionalized patients
Rights and dignity	true informed consent; a confidential diagnosis	respecting others' unpublished work	privacy of patient records

2. (example answer) I partly disagree. Researchers cannot control every use of their work, but the book lists misapplied results as a risk to society that researchers must weigh (see the MMR case). So they are responsible for reporting clearly and honestly and correcting errors they know about (8.10b), but not for misuse they could not foresee.
3. (example answer; check it against the documents) Similarities: all require voluntary, informed consent; all weigh risks against benefits; all let participants withdraw; all require minimizing harm; all expect research to have scientific or social value. Differences: origin (trials of Nazi physicians, 1947; US concerns after Tuskegee, 1978; a professional association); scope (Nuremberg covers

medical experiments, Standard 8 also covers animals, 8.09); form (10 principles; three broad principles; fifteen standards); of the three, only Belmont names justice as a principle; only Standard 8 covers scholarly integrity; Standard 8 allows consent to be dispensed with in some cases (8.05), whereas Nuremberg calls consent "absolutely essential".

4. (example answer) The stories could upset some participants, so consent must describe this discomfort and the right to decline or withdraw without consequences (8.02a). People with related personal experiences might be especially upset, so I would want a warning and a reminder that they can stop, as with the fear-of-crime survey in 3.4. If the purpose is withheld, the 8.07 conditions apply and it must be explained at debriefing (8.08); students taking part for credit need an alternative activity (8.04b). The risk seems minimal, so expedited review is probably enough.
5. (example answer) Follow Burger's approach: prescreen, stop well before any severe stress and remind participants that they can withdraw. Or use a mild task: an experimenter asks participants to continue a long, boring task (crossing out letters) after they want to stop, and obedience is how long they continue. The consent form describes the task, its risks (boredom) and the right to stop, and says the research question will be explained at the end.
6. See the Writing task.

3. Language A. (example answers) 1. You do not have to take part. If you start, you can stop at any time. 2. We will keep your signed form separate from your answers. Nobody will be able to connect your name to your answers. B. (example answer) First, identify and minimize the risks. Once you have done this, weigh the risks against the benefits. The next step is to create your consent and debriefing procedures, and then to get approval. Finally, follow through: stick to the protocol and monitor your participants.

My notes